

Bladder Care With a Nerve Condition

Patient Information · Options for spinal cord injury, MS, spina bifida and other nerve conditions

WARWIKI

Nerve conditions can change how the bladder fills and empties. It may squeeze too often, fail to empty, or hold urine under **high pressure**. High pressure can harm the kidneys without causing any symptoms. Good bladder care aims to **protect your kidneys**, keep you dry, prevent infections and fit your daily life.

Ways to Empty Your Bladder

Method	How it works	Good points	Downsides
Intermittent catheterization	You or a helper pass a thin catheter to empty the bladder, often 4–6 times a day, then remove it.	Nothing stays in the body. Often the first choice when you cannot empty.	Needs hand skill or a helper, and a schedule. Leaks between times can happen.
Suprapubic catheter	A tube through the lower belly into the bladder, draining to a bag. It is changed regularly.	Spare the urethra. Usually preferred over a urethral catheter for long-term use.	Needs a procedure to place. Infections and stones. The opening closes quickly if the tube falls out.
Urethral catheter	A catheter that stays in through the urethra, draining to a bag. Changed at least every 4 weeks.	Simple. Easy for caregivers.	Infections and stones. Over time it can damage the urethra or bladder neck.
Timed voiding	Urinating by the clock instead of waiting for an urge.	The most natural, when it is safe.	Only if tests show the bladder empties at a safe pressure.

Pushing on the belly or straining to empty can raise bladder pressure. Use it only if your team says your tests allow it.

Treatments That Calm the Bladder

- **Medicines** that relax the bladder muscle reduce urges and leaks. Some cause constipation, dry mouth or memory trouble.
- **Bladder Botox:** injections into the bladder wall through a thin scope, repeated when it wears off.
- **Sacral nerve stimulation:** a small implant that steadies bladder nerve signals. It can help some people, for example with MS, whose main problem is urgency. It is tested first, and is not usually advised after a spinal cord injury or with spina bifida.

Bladder Botox in Numbers

21 fewer

leaks a week with Botox, vs 9 fewer with a dummy injection (MS and spinal cord injury)

about 8 months

was the typical time before people asked for a repeat injection

about 1 in 3

who were not catheterizing had to start afterward

Learn the Terms

Neurogenic bladder

Bladder trouble caused by a nerve condition.

Catheter

A thin tube that drains urine from the bladder.

Suprapubic

Above the pubic bone, in the lower belly.

Urodynamics

A test that measures bladder pressure as it fills and empties.

Augmentation

Surgery that enlarges the bladder with a piece of bowel.

Autonomic dysreflexia

A sudden, dangerous rise in blood pressure after a spinal cord injury at T6 or above.

Numbers are from the main study. Your own results may differ; ask your team.

Autonomic dysreflexia: an emergency if your spinal cord injury is at T6 or above**Warning signs**

- Pounding headache
- Flushing or sweating above the injury; stuffy nose; blurred vision
- Pale, cool skin or goosebumps below the injury
- Blood pressure 20 or more points above your usual. Your usual may be low, so a "normal" reading can be high for you.

What to do now

- 1 **Sit up** and lower your legs. Loosen anything tight.
- 2 **Check the bladder first:** a kinked or blocked catheter, a full bag, or a missed catheterization. Drain it. Do not press or tap on the belly.
- 3 Check the bowel and skin. Check blood pressure every few minutes.
- 4 **Call 911** if it does not settle quickly. Carry a card that explains AD.

Surgery When Other Treatments Are Not Enough

- **Bladder augmentation** enlarges the bladder with a piece of bowel to lower pressure. It is major surgery: several days in the hospital and about 6 weeks of recovery. It needs lifelong yearly checks for mucus, stones and blood chemistry.
- **Catheterizable channel:** a small opening on the belly lets you catheterize from a wheelchair or when the urethra is hard to reach.
- **Urinary diversion:** urine drains to a bag on the belly when other options do not work.

Checkups That Protect Your Kidneys

- Kidney scans, a kidney blood test and sometimes urodynamics, on a schedule set by your risk (often every 1–2 years).
- Bacteria in the urine without symptoms are common with catheters and usually **should not** be treated.
- Tell your team about new leaks, more infections, blood in the urine or stones. Your plan may need to change.

Fewer Infections

- Empty on schedule and keep your bowel program regular.
- Coated (hydrophilic) catheters are linked to fewer infections.
- Daily antibiotics are usually not advised, unless infections keep coming back; ask your team. Cranberry has not been shown to prevent infections.

Call your care team if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- New back or side pain, or more spasms or leaks than usual
- Blood in the urine, or trouble passing a catheter

Go to an emergency department if:

- Signs of autonomic dysreflexia that do not settle
- A catheter that will not drain, or a suprapubic tube that falls out
- Fever with shaking chills, or you feel very unwell

Three Things to Remember

1. The main goal is to protect your kidneys, even when you feel well.
2. Choose an emptying method that fits your hands, your helpers and your day.
3. With an injury at T6 or above, a full bladder can trigger a blood-pressure emergency.